



Cover Note

Health AI Institute respectfully submits the attached comment in response to the HHS Request for Information, *Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care* (FR Doc. 2025-23641).

Our comments are informed by applied experience evaluating and implementing AI in clinical settings, convening stakeholders across the health care ecosystem, and leadership experience spanning federal health services research, clinical informatics, health system operations, and AI technology development. We focus on system-level actions HHS can take to reduce adoption barriers, align incentives, and support evidence-to-practice translation while maintaining patient safety and public trust.

We appreciate the opportunity to contribute and welcome continued engagement with HHS, ASTP/ONC, CMS, and FDA on these issues.

Submitted on behalf of:
Health AI Institute

Response to HHS Request for Information

Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care
(FR Doc. 2025-23641)

Submitted by: Health AI Institute

I. Perspective and Context

Health AI Institute appreciates the opportunity to provide input on how HHS can accelerate the adoption and effective use of artificial intelligence (AI) in clinical care. We support HHS's goal of enabling innovation while protecting patients, civil rights, and public trust.

Our perspective is grounded in applied, real-world experience. Health AI Institute engages across the health AI lifecycle, including evaluation of AI tools in clinical environments; convening health systems, payers, developers, patients, and regulators; and advising organizations responsible for governance and deployment. Our leadership and affiliates include former federal health services research leadership (including AHRQ), CMIOs, AI technologists, and health system executives responsible for implementation at scale. This vantage point allows us to observe not only whether AI works, but *why adoption succeeds or fails in practice*.

Across these engagements, we consistently find that AI adoption is constrained less by AI's technical capability than by misalignment among regulation, reimbursement, evidence generation, data infrastructure, and clinical workflows.

Health AI Institute

Website: <https://healthai.institute/>

LinkedIn: <https://www.linkedin.com/company/health-ai-institute>



II. Cross-Cutting Barriers to AI Adoption

- A. Regulatory uncertainty outside FDA-regulated medical devices:** While FDA has clarified pathways for AI-enabled medical devices, a growing share of AI used in clinical care falls outside traditional device definitions (e.g., documentation support, care coordination, operational decision support, and generative AI embedded in workflows). Across health systems we engage, legal and compliance teams frequently delay or block deployment not due to known safety risks, but due to uncertainty around accountability, liability, and post-deployment expectations. This uncertainty disproportionately affects smaller and resource-constrained organizations.
- B. Misaligned payment incentives:** In real-world implementations, AI often delivers value through cognitive efficiency, prevention, administrative burden reduction, and care coordination—outcomes that are weakly rewarded under fee-for-service payment. We repeatedly observe that the organizations investing in AI are not those capturing its financial benefits, undermining sustainability even when clinical value is evident.
- C. Evidence gaps focused on deployment, not accuracy:** The primary evidence gap is no longer whether AI models can perform discrete tasks, but whether they perform reliably, equitably, and safely over time and across settings. Health care organizations struggle with post-deployment monitoring, bias and drift detection, workflow integration, and economic evaluation.
- D. Interoperability that is not AI-ready:** Despite progress in data exchange, clinical data often lacks the provenance, context, labeling, and longitudinal linkage required for scalable AI evaluation and deployment. Interoperability that enables data movement but not data meaning limits AI impact.
- E. Patient trust and transparency:** Patients and caregivers express concerns related to transparency, escalation when AI is wrong, and potential inequities. Addressing these concerns is essential to durable adoption.

III. Strategic Recommendations for HHS

- A. Regulation: prioritize clarity and learning (ASTP/ONC, FDA):** HHS should emphasize regulatory clarity and predictability, particularly for non-device AI used in clinical care. This includes publishing cross-agency governance principles, clarifying post-deployment expectations, and supporting time-limited learning environments or safe harbors for well-scoped AI use cases. Reducing uncertainty will accelerate adoption while preserving patient protections.
- B. Reimbursement: align payment with value (CMS):** HHS should modernize payment policies to recognize AI as enabling clinical infrastructure when it demonstrably improves quality, access, or efficiency. High-value applications, including AI-enabled telehealth, remote monitoring, and wearables for chronic disease management, often generate value through predictive insights rather than encounters. Allowing AI costs within value-based models, supporting outcome-linked payment pilots, and enabling temporary reimbursement pathways would materially reduce adoption barriers.
- C. Research & development: refocus on evidence-to-practice translation:** HHS has an opportunity to rebalance AI research toward real-world implementation and learning. Priorities include post-deployment performance monitoring, workflow integration, organizational governance, and economic impact analysis. This learning-oriented approach reflects the legacy of federal health services research and learning health systems and is essential for AI in clinical care.

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- D. Interoperable, AI-Ready Data Infrastructure (ASTP/ONC):** Accelerating AI adoption requires interoperable data infrastructure beyond exchange alone. AI depends on longitudinal, context-rich data with consistent standards, provenance, and governance across care settings, including patient-generated data. HHS can catalyze progress by aligning interoperability policy with real-world AI use cases and supporting shared benchmarking and evaluation infrastructure.
- E. Evaluation and trust infrastructure:** Rather than certifying individual tools, HHS can catalyze adoption by supporting shared evaluation infrastructure, independent testing ecosystems, and accreditation aligned with federal principles. This approach promotes accountability without constraining innovation.

IV. Illustrative Learning-Oriented Federal Actions

The following actions illustrate non-prescriptive, learning-oriented approaches HHS could use to reduce risk and accelerate AI adoption in clinical care by aligning regulatory, payment, and evidence-generation levers.

- 1. National Clinical AI Sandbox Program:** A time-limited program enabling real-world deployment of AI tools in clinical settings under defined regulatory and reimbursement flexibility, paired with structured evaluation to generate evidence on safety, effectiveness, equity, and workflow integration. *(Primary beneficiaries: clinicians, health systems, regulators)*
- 2. AI Evaluation Commons:** A shared national resource supporting standardized benchmarks, AI-relevant data standards, post-deployment performance metrics, and failure-mode reporting, reducing duplication across health systems and enabling more transparent, comparable AI evaluation. *(Primary beneficiaries: health systems, developers, researchers)*
- 3. Temporary AI Payment Pathways:** Time-limited reimbursement pathways for high-evidence AI tools across care settings within value-based care models, allowing real-world performance, equity, and cost impacts to be evaluated prior to permanent coverage or payment decisions. *(Primary beneficiaries: patients, clinicians, value-based care organizations)*
- 4. Organizational AI Readiness Grants:** Targeted support for health care organizations to build governance structures, clinician training, change management capacity, and human-AI workflow integration necessary for safe and effective AI adoption. *(Primary beneficiaries: clinicians, health system leadership)*
- 5. Patient Trust and Transparency Initiative:** Development of patient-centered norms and tools for disclosure, consent, escalation, and communication when AI is used in clinical care, addressing patient concerns while strengthening public trust and accountability. *(Primary beneficiaries: patients and caregivers)*

V. Conclusion

AI can improve outcomes, reduce burden, and enhance productivity across the health care system. Achieving this potential will require coordinated federal action that aligns regulation, reimbursement, research, and trust infrastructure with the realities of clinical care.

Health AI Institute appreciates HHS's leadership and welcomes continued engagement to advance these shared goals.

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About the Health AI Institute (HAI)

- The [Health AI Institute \(HAI\)](https://healthai.institute/) provides a nonpartisan, nonpolitical platform where business leaders, technology experts, and industry partners can collaborate with policymakers to develop AI strategies that drive innovation while ensuring responsible governance. Our mission is to foster interdisciplinary collaboration, drive AI-powered innovation, and shape the future of medicine by addressing the most pressing challenges in healthcare. We envision a future where AI-enabled technologies enhance patient care, improve efficiency, and promote health across the U.S. and beyond.



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